

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 1

New Patient: Meera (28 y/o, Female Teacher, Kerala)

Full Detailed – Realistic Style

SESSION INFORMATION

Client:	Meera (28 years, Female)	Therapist:	Dr. A.
Session #:	1 of 16–18 planned	Presenting Issue:	Moderate Depression + Anxiety

PRE-SESSION THERAPIST NOTES

New client. 28-year-old high school teacher from Kerala. Presenting with 8 months of increasing low mood, fatigue, loss of interest in teaching and hobbies, sleep disturbance, and anxiety about being judged by colleagues and students. She continues to work but feels emotionally numb and exhausted. Self-referred after a colleague suggested therapy. BDI-II = 23, GAD-7 = 14. No previous therapy. Risk appears low. Plan: Build rapport, understand presenting problems, gently introduce CBT model, set initial goals, and assign simple monitoring homework.

SESSION START – BUILDING RAPPORT

Therapist: Good morning, Meera. Thank you for coming in today. I know starting therapy can feel like a big step. How are you feeling about being here?

Client: A bit unsure actually. I've been feeling this way for months, but I don't know if talking about it will really change anything. I'm just very tired all the time.

Therapist: That's completely understandable. Many people feel unsure or skeptical in the beginning, especially when they've been struggling for a while. Today we'll just focus on understanding what's been going on for you. There's no pressure. Everything stays confidential.

Client: Okay... thank you.

PRESENTING PROBLEMS

Meera described an 8-month period of gradually worsening low mood, fatigue, and loss of interest in teaching and previously enjoyed activities (reading, spending time with friends). She feels emotionally numb most of the time and has to push herself to get through the school day.

She experiences anxiety about being judged by colleagues and students, especially when she feels she is not performing at her best. Sleep is disturbed — she has difficulty falling asleep and often wakes up early feeling worried or empty.

She has become more withdrawn from colleagues and friends. She reports feeling like she is “just going through the motions” and questions whether she is a good teacher anymore. She has no history of self-harm or suicidal ideation.

BRIEF HISTORY

Symptoms gradually increased over the last 8 months, coinciding with increased workload at school and some difficult parent-teacher interactions. She had one previous period of low mood during her B.Ed. studies, which improved after exams ended. No history of trauma or substance use. She describes herself as someone who has always been conscientious and puts a lot of pressure on herself to be a “good teacher.”

INTRODUCTION TO CBT MODEL

Therapist: From what you've shared, it sounds like you've been carrying a lot for several months and it's become very heavy. One way we look at this in CBT is by understanding the connection between our thoughts, feelings, and behaviors. Would it be okay if I showed you with one of your recent experiences?

Client: Okay...

Therapist: Can you tell me about a recent situation where your mood was particularly low or you felt anxious?

Client: Last week I had a parent-teacher meeting. I spent the whole day before worrying that the parents would think I'm not good enough. After the meeting, I felt completely drained and just wanted to sleep.

Therapist: And during that time, what kind of thoughts were going through your mind?

Client: I kept thinking “They're going to see that I'm not a good teacher. Everyone will realize I'm struggling.”

Therapist: And when you had those thoughts, how did you feel emotionally and physically? What did you end up doing?

Client: I felt very anxious and ashamed. My chest felt tight. After the meeting, I went home and just lay on the bed for hours. I didn't even eat dinner.

Therapist: So we can see how those thoughts led to strong anxiety and shame, which then led to withdrawing and having very low energy afterward. This is the kind of cycle we often work on in CBT. Does this way of looking at it make sense to you?

Client: Yeah... I can see how my thoughts made everything feel worse.

COLLABORATIVE GOAL SETTING

Therapist: If therapy is helpful, what would you like to be different in the next few months?

Client: I want to feel like I have some energy again. I want to stop feeling so numb. I want to feel like I'm actually a decent teacher instead of constantly doubting myself.

Therapist: Those are very meaningful goals. We can work on making them more specific as we go forward.

HOMEWORK ASSIGNMENT

Therapist: For this first week, I'd like you to start a simple daily diary. It only takes a few minutes. Just note:

1. Time of day and a brief situation
2. Your mood rating from 0 to 10
3. What you were doing
4. Any strong thoughts that came up

This will help us understand the patterns more clearly. Do you think you can try this?

Client: I can try... but I'm not sure I'll have the energy to do it every day.

Therapist: That's completely fine. Just do it whenever you can. Even a few entries will be helpful. We'll review it next session.

SESSION SUMMARY & FEEDBACK

Therapist: Today we talked about what's been going on for you, looked at how thoughts, feelings, and behaviors are connected, and set some initial goals. We also agreed on the first homework. How are you feeling about today's session?

Client: It was okay. I still don't know if this will really help, but I'm willing to try.

Therapist: That's fair. We'll take it step by step. Any questions before we finish?

Client: Not really.

POST-SESSION THERAPIST NOTES

- Client presented with clear symptoms of moderate depression with anxiety features.
- Some initial skepticism and low energy/motivation expressed (realistic).
- Client was able to identify thoughts-feelings-behaviors connection with gentle guidance.
- Homework assigned with realistic client hesitation about energy to complete it.
- Risk: Low. Next session: Review homework diary, deepen formulation, identify key negative automatic thoughts.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.